

Activity 6 — Divergent Ideation with GenAI (Breadth Before Judgement)

Meridian Health — solving the retention root cause

Topic	Topic 2
Objective	A5 · K4 — Generate a wide solution set using appropriate problem-solving techniques.
Duration	45 minutes
Grouping	Teams of 3-5
Tools	ChatGPT / Copilot / Gemini, Pinboard
Ed-tool	Pinboard — https://alfredang.github.io/pinboard/

The Scenario

Your 5 Whys in Activity 2 established the root cause at Meridian Health: a workforce retention and onboarding failure that leaves 40% of the 07:00 roster staffed by untrained relief phlebotomists at 14 minutes per draw instead of 6.

The Clinical Director has accepted the analysis and killed the S\$1.2m seventh-centre proposal. She now wants options — and she has been explicit: "Don't bring me three sensible ideas. Bring me twenty, including the ones that sound ridiculous, and then tell me which four are real."

Budget guidance: up to S\$250,000 over 12 months. Constraint: MOH clinical protocols on phlebotomy competency cannot be relaxed under any circumstances.

The Evidence

Meridian Health — solution constraints and context

Constraint	Detail
Budget	Up to S\$250,000 over 12 months
Clinical	MOH phlebotomy competency standards are non-negotiable
Contract	07:00-09:30 fasting draw window is contractual with corporate clients
Courier	Lab courier at 09:45 is fixed by the external lab partner
Workforce	Permanent phlebotomist turnover 44%/yr; relief pool is agency-supplied
Training	Current onboarding is 2 days shadowing, no structured sign-off
Market	Phlebotomist salaries are within 5% of the market median
Shift	Early shift (06:30 start) attracts no differential pay

Discussion Questions

1. Your team generated ideas manually first, then with GenAI. Compare the two lists — how many did the AI produce that your team would never have said out loud, and why not?
2. The AI was asked for two ideas borrowed from another industry. Which cross-industry idea is most transferable to Meridian, and what would have to be true for it to work?
3. Apply SCAMPER's 'Reverse' and 'Eliminate' to the fixed 07:00-09:30 window. Which supposedly fixed constraint turns out to be an assumption rather than a real constraint?
4. Identify one idea on your list that is genuinely ruled out by the MOH clinical constraint. How do you tell a hard constraint from a habit?
5. Cognitive fixedness is the tendency to reach for the familiar solution. Find one idea on your list that only appeared because you deliberately broke fixedness — what triggered it?

Step-by-Step

6. Restate the Meridian root cause from Activity 2 in one sentence so the whole team is solving the same thing.
7. MANUAL ROUND FIRST — set a 7-minute timer. Each person writes ideas silently on post-its. No discussion, no evaluation, no 'that won't work'.
8. Post all ideas to the wall at <https://alfredang.github.io/pinboard/> and count them.
9. Now run the GenAI divergent ideation prompt from the slide.
10. Add every AI idea your team did not already have to the board, in a different colour.
11. Count the two groups. Discuss: which AI ideas would nobody in your team have proposed in a real meeting, and what stopped them?
12. Run the SCAMPER section of the prompt against the '07:00-09:30 at the centre' assumption.
13. For each SCAMPER letter, add the resulting idea to the board.
14. Go through the board and mark each idea HARD CONSTRAINT (genuinely ruled out by MOH or contract) or HABIT (only feels fixed).
15. Do NOT rank or cost anything yet — carry the full board forward to Activity 7.

The GenAI Prompt

Act as an innovation consultant running a divergent ideation session. Do NOT evaluate or filter yet.

Root cause to solve: A Singapore health-screening operator has 44% annual turnover among permanent phlebotomists. Vacancies are backfilled by agency relief staff who take 14 min per blood draw versus 6 min for trained staff. 40% of the 07:00 roster is relief staff, causing 31% of morning appointments to overrun and 23% of samples to miss the fixed 09:45 lab courier.

Constraints: S\$250k over 12 months; MOH phlebotomy competency standards cannot be relaxed; the 07:00-09:30 fasting window and the 09:45 courier are both fixed.

Generate 20 distinct solutions grouped under:

- People & Retention
- Training & Onboarding
- Process & Scheduling
- Technology & Automation
- Customer/Client Experience

Rules:

- Include at least 4 ideas that would normally be dismissed as unrealistic
- Include at least 2 ideas borrowed from a COMPLETELY different industry (say which)
- One line each, phrased as an action
- Do NOT rank, cost or evaluate them

Then, separately: apply SCAMPER (Substitute, Combine, Adapt, Modify, Put to another use, Eliminate, Reverse) to the single assumption that 'every draw must be done by a phlebotomist at the centre between 07:00 and 09:30' and give one idea per SCAMPER letter.

Trainer Debrief

Expected answers and the points to draw out. Trainer reference.

EXPECTED RANGE — a strong list spans all five groups and includes at least these shapes:

PEOPLE & RETENTION — early-shift differential pay; career ladder to senior phlebotomist; convert best relief staff to permanent; retention bonus at 12 and 24 months; exit-interview programme to find the real reason people leave.

TRAINING & ONBOARDING — structured 10-day onboarding with competency sign-off; simulation arm practice before live draws; buddy system pairing relief with permanent staff; a micro-credential the agency pool can earn before their first shift.

PROCESS & SCHEDULING — stagger appointment slots so relief staff take simpler draws; roster all 5 permanent staff at 07:00 and taper later; pre-screen difficult-draw patients to trained staff; negotiate a second courier pickup.

TECHNOLOGY — vein-finder devices to cut the 18% repeat rate; digital pre-registration; automated fasting-compliance reminders the night before; queue app.

CLIENT EXPERIENCE — corporate on-site draws at the client's office; extend the window by agreement; tiered booking so the largest client gets trained staff.

KEY TEACHING POINTS:

1. **DIVERGENCE MUST PRECEDE JUDGEMENT.** Teams that evaluate while generating typically stop at 6-8 ideas, all conventional. The instruction to include 'ridiculous' ideas exists because the ridiculous ones are where fixedness breaks — and the merely unusual ones next to them are often viable. Count your team's manual list against the AI list; the gap IS the lesson.
2. **GENAI IS A DIVERGENCE ENGINE.** This is its strongest contribution to problem solving: breadth at near-zero cost, free of the political self-censorship that stops a junior employee proposing 'pay the early shift more' in front of a Finance Director. The AI has no career risk. Note that it also has no judgement — which is Activity 7's job.
3. **THE SCAMPER REVEAL:** the biggest unlock is that the 07:00-09:30 window is fixed only because draws happen AT THE CENTRE. 'Reverse' (bring the draw to the client's office) and 'Eliminate' (remove the centre visit for corporate clients entirely) both dissolve the constraint rather than working within it. Meanwhile the MOH competency standard is a genuine hard constraint — it is a regulation, not a habit. Teaching learners to separate REGULATION from HABIT is the durable skill here.
4. **CROSS-INDUSTRY TRANSFER:** mobile phlebotomy mirrors mobile blood-donation drives; the buddy/competency-card model comes from aviation and F&B; surge differential pay comes from ride-hailing. Analogy is a recognised problem-solving strategy, and GenAI is unusually good at it because it has read across every domain.
5. **Do NOT let teams rank yet.** Ranking is Activity 7. Holding the line between divergence and convergence is itself the discipline being taught.

Self-Check — Is Your Output Finished?

Your divergence is sufficient when: you have at least 20 distinct ideas spanning all five groups; at least 4 would raise an eyebrow in a management meeting; at least 2 are borrowed from another industry and you can name it; you have one idea per SCAMPER letter; and you

have correctly separated at least one HARD CONSTRAINT from at least one HABIT. If every idea on your board is comfortable, you have not diverged — you have listed.